



# Armada Kalamunda Group

## Interpreter and Translator Guideline

### 1. Purpose

The purpose of this guideline is to outline the requirements for interpreter and translator services at Armada Kalamunda Group (AKG).

### 2. Applicability

This guideline applies to all services throughout the Armada Kalamunda Group which incorporates the Armada Health Service, Kalamunda Hospital and Orchard Avenue Centre.

### 3. Guidelines

Language or culture should not be a barrier to services.

- All persons attending AKG have the right to access culturally appropriate health care regardless of cultural origin, linguistic skills and religious belief.
- All consumers and carers who need assistance with English or who are hearing impaired are to be provided with access to appropriate interpreting and translating assistance.

AKG complies with the [Western Australian Language Services Policy 2020](#), the Department of Health Western Australia (HDWA) [Language Services Policy MP 0051/17](#) and the East Metropolitan Health Service (EMHS) [Cultural considerations when caring for Aboriginal People Policy EMHS:35](#)

Staff must be aware of and respect the patients/clients cultural, linguistic and religious requirements.

The need for an interpreter is to be identified and communicated to clinical staff prior to attendance where possible, or ASAP on attendance.

The requirement for an interpreter and language must be recorded in the following places:

- appropriate history/assessment/referral form
- on the Patient Administration System (PAS).

It is the responsibility of all staff to update relevant triage / admission medical record forms when a patient requires an interpreter. The label below is approved to flag this requirement:



## 4. Consent

A patient from a culturally and/or linguistically diverse background or who has a physical impairment (such as blindness), or mental impairment, may need additional support to provide valid consent.

Where a patient is identified as having special needs or language requirements, the health professional should engage the services of a competent interpreter to assist the patient to understand the procedure and relevant implications.

Interpreters from an accredited service must be used to obtain consent for patients/clients with special needs or language requirements, as identified in the [Department of Health Consent to Treatment Policy MP 0175/22](#)

**Note:** For a high-risk communication where the higher level of interpreter or translator is not available, consideration should be given to rescheduling the appointment. The decision must take into account clinically appropriate timeframes and consumer needs and risks.

### **Refer to Appendix 2: When to use an Interpreter Decision Making Tree**

**Note:** Family members, in particular spouses and children, should not be used as a substitute for a competent interpreter. Persons under the age of 18 years must not be used as interpreters or translators unless they are competent interpreters or translators.

**Note:** Bilingual staff must not be used in place of a competent interpreter/translator.

Contact numbers for interpreter services must be clearly displayed and translated information must be available for the public where the need is identified.

If a patient / consumer declines the use of an interpreter, staff are to document the details of the decision and circumstances for not using an interpreter in the Digital Medical Record (DMR).

## 5. Medical emergencies

In the case of a medical emergency where urgent treatment/care is required to save a patient's/client's life or prevent serious harm and a competent interpreter cannot be obtained, a family member or friend **over the age of 18 years** or bilingual health professional may act as an interpreter to obtain information for immediate diagnosis or treatment.

Assistance from a person under 18 years of age must **only** be considered when an adult family member or friend or bilingual health professional is not available.

A competent interpreter must be engaged as soon as possible for the ongoing diagnosis and treatment of the patient.

## 6. Documentation

Clinical staff are responsible for ensuring clear documentation of the following within the DMR:

- If an interpreter is required
- Confirmation that interpreter has been booked
- If an interpreter is not available. Document other options considered (see appendix 2– Decision Making Tree).
- When the interpreter failed to attend appointment or was late. Document any correspondence with the agency and outcome.

- Reporting concerns regarding interpreter's competency, accuracy or conduct while in the workplace to the Service Provider.
- When patient/client or patient's/client's family decline the offer of an interpreter.
- Language label to be added to relevant triage/admission documents as well as the inside cover of the patients' health record.

Clerical staff are to action:

- Recording of language information in webPAS

## **7. Interpreter services available:**

WA Health utilises the following Translating / Interpreting services:

### **7.1 Translating Interpreting Service (TIS)**

- Email the TIS Request for On-Site Interpreting form to [tis@immi.gov.au](mailto:tis@immi.gov.au) or, phone 131450

### **7.2 On Call Interpreters and Translators Agency**

- Book online Australian Government - [Translating and Interpreting Service](#) or phone 92257700

### **7.3 Access Plus WA Deaf**

- Telephone the [NDIS – Access Interpreting](#). Monday – Friday 9 am – 5 pm on 9441 2677. After-hours and Emergencies on 0410 017 540.
- [Interpreter Request Form](#)

### **7.4 Aboriginal Interpreting Services**

- Contact the AKG Aboriginal Liaison Officer
- Refer to the resources on [Aboriginal Interpreting WA](#) or [book online](#)

### **7.5 WA Interpreters**

- Telephone: 9361 3248 Monday to Friday 0700 am – 1730 pm

### **7.6 Additional interpreting resources**

If the service required is not available or unable to be located, refer to the [Policy Framework Mandatory Policy Template \(health.wa.gov.au\)](#) MP 0051/17 and the [Interpreting and Translating Services CUAITS2017 \(www.wa.gov.au\)](#)

For languages of some Aboriginal and new and emerging communities / languages, a tertiary qualification or NAATI certification may not be available. It is the Service Provider's responsibility to provide the highest qualification/certification, however, in some instances the provision of a non-certified / credentialled interpreter may be the only option and is acceptable given the risks associated with not providing language / interpreter support.

Any concerns regarding the suitability of the interpreter should be provided to the Service Provider by clinic staff.

Web-based translation applications must not be used as they cannot be relied upon for accuracy. Incorrect translations could pose a risk to consumer rights, health and safety.

## 8. Booking of an interpreter / translator

Interpreters are routinely booked between 0700 to 1800 hours to prevent penalty charges from occurring. Prior to booking, the staff member will confirm:

- Appointment duration
- Gender specific requirements; i.e.; male or female interpreter required.
- Complete an appropriate Interpreter Service Provider booking form at least 24 hours prior to the need (except in emergency situations when phone interpreter can be booked).

When Booking provide the following information:

- Name, title and telephone extension of the staff member who is making the booking;
- Language/s /dialect required; (Please refer to Western [WA Language Services Policy 2020](#) for language options)
- Patient's full name and gender;
- Date of birth;
- Unit Medical Record Number (UMRN);
- Time and place of appointment;
- Request for particular gender

Refer to Appendix 1: [Booking an Interpreter](#)

### 8.1 Urgent bookings

Where there is an urgent request for a telephone interpreter ensure that the interpreter service providers are aware of:

- the nature of the emergency;
- the contact details of the staff member and supervisor;
- confirm the contact details by email or facsimile.

## 9. Working with interpreters

For information on how to work with an interpreter refer to the Office of Multicultural Interests – [How to engage interpreting services.](#)

## 10. Reconciliation of interpreter invoices

All invoices are sent to the Business Support Officer in the first instance. The Business Support Officer will forward the invoices to relevant departments for processing.

Staff responsible for payment of invoices are to confirm the service took place by reconciling green service receipt to invoice. If the green service receipt is unable to be located, staff must review patient's medical record to ensure the interpreting service was used as indicated by invoice.

Once service has been confirmed, payment can be processed via i-Proc.

All invoices must be kept in a safe place for view and reference in accordance with the HDWA [Financial Management Framework](#).

## 11. Legislative context

The following documents are required to give effect to this policy:

- Department of Health Western Australia (HDWA) [Financial Management Framework](#)
- Department of Health Western Australia (HDWA) [Language Services Policy MP 0051/17](#)
- East Metropolitan Health Service (EMHS) [Cultural considerations when caring for Aboriginal People Policy EMHS:35](#)

## 12. Supporting information

The following documents support the implementation of this policy:

- Department of Health Western Australia (HDWA) [Language Services Policy MP 0051/17](#)

## 13. Compliance, monitoring and evaluation

Compliance against this guideline will be monitored by clinical staff utilising interpreters and Unit managers reviewing and approving interpreter invoices.

## 14. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.3 Healthcare rights and informed consent
- The health service organisation has a charter of rights that is:
  - a. Consistent with the Australian Charter of Healthcare Rights<sup>53</sup>
  - b. Easily accessible for patients, carers, families and consumers
- Standard 2 – Action 2.5
- The health service organisation has processes to identify:
  - a. The capacity of a patient to make decisions about their own care
  - b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves
- Standard 2 – Action 2.8 Communication that supports effective partnerships
- The health service organisation uses communication mechanisms that are tailored to the diversity of the consumers who use its services and, where relevant, the diversity of the local community

## 15. Document owner

### Manager Social Work

Enquiries relating to this guideline may be directed to:

Title: Manager Social Work  
Department: Social Work  
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## 16. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

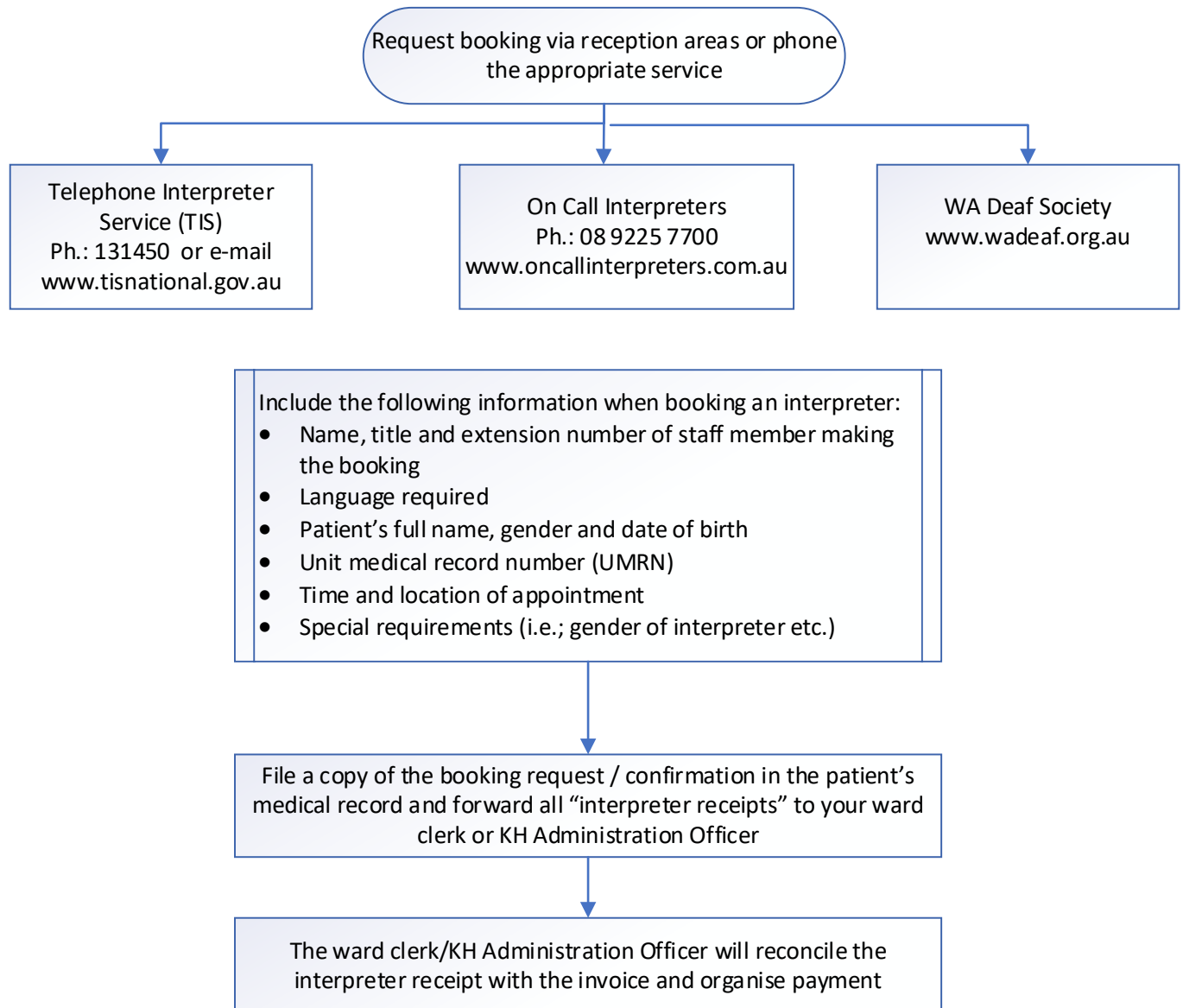
| Version | Effective from | Effective to | Amendment(s)     |
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| V4      | 24/02/2025     | 24/02/2028   | Scheduled Review |

## 17. Authorisation

This guideline has been authorised and issued by the Director Operations & Corporate Services

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|--------------------|-----------------------------------------------------------|
| Authorised by      | Michael Morton – Director Operations & Corporate Services |
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## Appendix 1: Booking an Interpreter



## Appendix 2: When to use an Interpreter – Decision Making Tree

